

King Fahad Central Hospital

Document ID: SU-009-PROT-01 | Parent Policy: SU-009 — Stroke Performance Improvement, Data Registry and Peer Review |
Version: 1.0 | Controlled Copy

Stroke Registry Policy

Case Ascertainment, Data Abstraction & Reporting — Aligned with AHA/ASA Get With The Guidelines—Stroke and Joint Commission Stroke Center Certification Requirements

1. Purpose

This policy defines how stroke and TIA cases are identified, abstracted, validated, and submitted to the hospital's stroke registry, and how registry data are used for internal performance improvement and external certification reporting. Consistent, complete, and timely registry participation is a foundational requirement of AHA/ASA Get With The Guidelines—Stroke and Joint Commission (or equivalent) stroke center certification.

2. Registry Program

- ☐ Primary registry platform: AHA/ASA Get With The Guidelines—Stroke (GWTG-Stroke) — specify institutional registry platform/vendor: _____
- ☐ State or regional registry participation (if applicable, e.g., state stroke registry, CDC Paul Coverdell Program): _____
- ☐ Registry supports data submission for Joint Commission (or applicable accrediting body) Primary Stroke Center / Comprehensive Stroke Center / Thrombectomy-Capable Stroke Center / Acute Stroke Ready certification, as applicable to this facility's designation

3. Case Ascertainment

3.1 Inclusion Criteria — Cases to be Captured

- ☐ All patients with a final diagnosis of acute ischemic stroke
- ☐ All patients with a final diagnosis of transient ischemic attack (TIA)
- ☐ All patients with a final diagnosis of intracerebral hemorrhage (ICH)
- ☐ All patients with a final diagnosis of subarachnoid hemorrhage (SAH)
- ☐ Patients transferred in or out for stroke-related care, per registry transfer-case rules
- ☐ Both admitted and — per registry/certification definition — applicable ED-treat-and-release or ED-death cases as required by the certifying body

3.2 Concurrent Case Identification

- ☐ **Stroke Program Coordinator (or designee) performs concurrent (real-time/daily) case identification via stroke alert log, ED stroke activations, and nursing unit stroke admission notifications**
- ☐ Concurrent identification is the primary case-finding method — relying solely on retrospective coding/billing data as the primary method does not meet consecutive case ascertainment expectations

3.3 Retrospective Validation

- ☐ Monthly (or per policy interval) cross-check of concurrent case log against ICD-10-CM discharge diagnosis codes / administrative data to identify any missed cases
- ☐ Discrepancies (cases found in coding data but not in concurrent log) investigated and added to registry; root cause of the miss documented

Joint Commission and similar certifying bodies expect a high rate of consecutive case ascertainment (commonly cited target $\geq 90\%$) — confirm the exact current threshold with the certifying body's published manual.

4. Data Abstraction

4.1 Timeline

Activity	Target Timeframe
Case flagged/logged for registry	Within 24–72h of admission (concurrent identification)
Data abstraction completed	Per registry submission deadline — commonly within 30–45 days of discharge; confirm current registry-specific deadline
Data entered/submitted to registry platform	Per registry submission deadline; batch or rolling submission per institutional workflow
Quarterly performance report generated	Within [specify] days after quarter close

4.2 Roles

- ☐ Stroke Program Coordinator / Data Abstractor(s): primary responsibility for chart abstraction, data entry, and record completeness
- ☐ Stroke Medical Director: reviews outlier cases, quality concerns, and signs off on performance improvement actions arising from registry data
- ☐ Health Information Management / Coding: supports ICD-10 code cross-check and administrative data pulls for retrospective validation
- ☐ IT/Informatics: supports data extraction interfaces, registry platform access, and data security
- ☐ Nursing units / ED: support timely and accurate concurrent documentation (LKW, NIHSS, treatment times) that abstractors depend on

4.3 Core Data Elements Abstracted

- ☐ Demographics and arrival information (mode of arrival, transfer status, arrival time)
- ☐ Last known well time and symptom discovery time
- ☐ NIHSS on arrival (and at other required intervals)
- ☐ Imaging times (door-to-CT, door-to-interpretation) and key findings (ASPECTS, occlusion site, hemorrhage)
- ☐ Treatment given: IV thrombolysis (dose, times), mechanical thrombectomy (puncture/recanalization times, mTICI score), or neither, with documented reason if not given
- ☐ Core measure elements: dysphagia screening, VTE prophylaxis, antithrombotic therapy, statin therapy, anticoagulation for atrial fibrillation, stroke education, rehabilitation assessment, smoking cessation counseling
- ☐ Discharge disposition, discharge medications, and follow-up plan
- ☐ Functional outcome measures (e.g., modified Rankin Scale) at discharge and/or follow-up, per registry requirement
- ☐ In-hospital complications and mortality, as applicable

5. Data Quality & Validation

- ☐ Periodic (e.g., quarterly) inter-rater reliability check: second abstractor independently re-abstracts a sample of charts and results compared for consistency
- ☐ Data completeness audit: percentage of required fields populated per submission cycle reviewed by Stroke Program Coordinator
- ☐ Outlier and "failure" cases (e.g., measure not met, unexpected time delay, adverse event) flagged for case review at Stroke Committee/PI meeting
- ☐ Data discrepancies between registry entry and source medical record corrected and documented with reason for correction

6. Performance Improvement Loop

- ☐ Registry-derived performance measure results reviewed at Stroke Committee meetings per defined cadence (e.g., monthly or quarterly)
- ☐ Measures falling below internal or certifying-body threshold trigger a documented performance improvement (PI) action plan
- ☐ PI action plans include: root cause analysis, assigned owner, corrective action, timeline, and re-measurement plan
- ☐ Benchmarking against state/national GWTG-Stroke aggregate data and peer/award recognition thresholds (as applicable) reviewed periodically
- ☐ Registry data used to support annual stroke program evaluation required for certification renewal

7. Confidentiality & Data Security

- ☐ Registry data handled per institutional HIPAA/privacy policy; access limited to authorized stroke program, quality, and coding personnel
- ☐ Data transmitted to external registry platforms via secure, institutionally approved methods only
- ☐ Patient-identifiable information removed/limited per registry data-use agreement and minimum-necessary standard
- ☐ Registry data used for quality improvement purposes per applicable peer-review/quality-assurance legal protections — confirm scope with institutional legal/compliance counsel

8. Certification & Reporting Requirements Supported

- ☐ Quarterly (or per certifying-body schedule) data submission to support Joint Commission / state / other applicable stroke center certification
 - ☐ Minimum case volume and data completeness thresholds tracked per the facility's specific certification level (Acute Stroke Ready, Primary, Thrombectomy-Capable, or Comprehensive Stroke Center)
 - ☐ Registry participation and performance data available for on-site survey/review upon request
- Confirm the exact current volume, completeness, and submission-frequency requirements against the certifying body's current manual, as these are updated periodically and vary by certification level.*

9. Registry Program Quality Metrics (Tracked Internally)

- ☐ Percentage of eligible cases captured concurrently (consecutive case ascertainment rate)
- ☐ Percentage of cases with complete required data fields
- ☐ Timeliness of data abstraction and submission relative to deadline
- ☐ Number of discrepancies identified and corrected per audit cycle
- ☐ Number of PI action plans opened/closed related to registry findings

Policy Approval

Stroke Medical Director: _____ Date: _____

Stroke Program Coordinator: _____ Date: _____

Quality/Performance Improvement Director: _____ Date: _____

Next Scheduled Policy Review Date: _____

References: AHA/ASA Get With The Guidelines—Stroke program requirements and data dictionary; American Stroke Association International Stroke Center Certification program Disease-Specific Care/Stroke Certification program manuals (Primary, Comprehensive, Thrombectomy-Capable, Acute Stroke Ready); CDC Paul Coverdell National Acute Stroke Program; applicable state stroke registry requirements. This template must be re-verified against the current registry data dictionary and certifying-body manual at the time of institutional adoption, as data elements, thresholds, and submission timelines are updated periodically.